Fax from: Banner Health

Providers and/or staff with Clinical Connectivity access
Please use the link below for future continuing care requests. If your office does not have access or if you need to update your access, this link will walk you through the process.

https://access.bannerhealth.com/clinicalconnectivity/

The following information is intended only for the use of the individual or entity to which it is addressed and contains information that is privileged, confidential and exempt from disclosure under applicable law. If the reader of this message is not the intended recipient or employee/agent responsible for delivering the message to the intended recipient, you are hereby notified that any dissemination, distribution or copying of the communication is strictly prohibited. If you receive this communication in error, please contact:

BANNER HEALTH SUPPORT DESK
602-747-4444

For Substance abuse Cases: This information has been disclosed to you from records protected by Federal Confidentiality Rules (42 CFR Part 2). The federal rules prohibit you from making any further disclosure of this information unless further disclosure is expressly permitted by the written consent of the person to whom it pertains or is otherwise permitted by 42 CFR Part 2. The general authorization for the release of medical and other information is not sufficient for this purpose. The federal rules restrict the use of the information to criminally investigate or prosecute any alcohol or drug abuse patient.

For Communicable Disease cases: This information is provided to you from confidential records which are protected by Arizona State Law that prohibits further disclosure of this Information without the specific written consent of the person to whom it pertains, or as otherwise permitted by law. A.R.S. Section 36-664 (f) (G).

Banner Health.

May 22, 2026

Bodycentral Physical Therapy - Broadway Blvd
6303 E. Broadway Blvd
Tucson, AZ 85710

Re: TESSA VILLARREAL
DOB: 05/25/2000

Dear Colleague,

Referral Details
Medical Service: Physical Therapy
Referral Type: Evaluate and Treat
Referral Reason: 31.4 weeks gestation with sciatica pain
Codified Reason: M54.30 - Sciatica
Refer From BUMT North Campus OPC - OBGYN/AZT NC OBGYN - MCDANIEL NP, ERIKA
Written Date: 05/22/2026
Requested Start Date: 05/22/2026
Refer To: Bodycentral Physical Therapy - Broadway Blvd -
Priority: Urgent
Instructions to Staff: NAR/NRR 97162. AUTH FOR THERAPY SESSIONS MUST BE OBTAINED BY SERVICING PROVIDER

Insurance
Health Plan: BCBS OUT OF STATE
Payer: BCBS OUT OF STATEs
Financial Class: HMO PPO EPO
Subscriber: HAMMER, ROBBIN
Member Number: OER201148335
Begin Date: 04/16/2026

Authorization
Authorization Number: NAR CONSULT

Authorization Type: Professional Authorization
Authorization Status: No Action Required
Begin Date: 05/22/2026
End Date: 05/22/2027

Other Insurances
Health Plan: TRICARE WEST TRIWEST
Payer: TRICARE WEST TRIWEST
Financial Class: TriCare Champus VA
Subscriber: VILLARREAL, TESSA DIANE
Member Number: 02261091601
Group Number: 0
Begin Date: 02/01/2026

Sincerely,
JoAnn Gomez

The following document(s) were included in the letter:
May 22, 2026 14:20:29 MST - (05/22/2026) Obstetrics Prenatal Office Visit Note

Result type:	OB/GYN Office/Clinic Note
Date/Time of Service:	May 22, 2026 14:20 MST
Result status:	Auth (Verified)
Result title:	Obstetrics Prenatal Office Visit Note
Contributed By:	MCDANIEL NP, ERIKA on May 22, 2026 15:35 MST
Verified by:	MCDANIEL NP, ERIKA on May 22, 2026 15:35 MST
Encounter info:	000011398823, AZT NC OBGYN, Prenatal, 05/22/2026 -
Patient Info:	VILLARREAL, TESSA DIANE, 05/25/2000, Female

* Final Report *

Chief Complaint
ROB 32 WEEKS 4 DAYS EDD 07/20/26

Patient Information
Name: VILLARREAL, TESSA DIANE Age: 25 Years DOB: 05/25/2000 Sex: Female

Date of Service: 05/22/2026 14:20 MST

Prenatal Information
Gestational Age (EGA) and EDD * Note: EGA calculated as of 05/22/2026

EDD: 07/20/2026 EGA*: 31 weeks 4 days
Type: Authoritative Method Date: 10/13/2025

Method: Last Menstrual Period (10/13/2025)
Confirmation: Confirmed
Description: --
Comments: --
Entered by: NAPPER CNM, AMANDA on 03/24/2026

Other EDD Calculations for this Pregnancy:

Method: Ultrasound
Method Date: 04/17/2026
EDD: 07/15/2026
EGA (At Entry): 27 weeks 2 days
Type: Non-Authoritative
Comments: --
Entered by: NAPPER CNM, AMANDA on 05/19/2026

Prenatal Exam and Notes
Weight: 110.4 kg
Systolic Blood Pressure: 112 mmHg
Diastolic Blood Pressure: 75 mmHg
Pregnancy History **G1 P0(0,0,0,0)**

No previous pregnancies history have been recorded
Risk Factors (Current Pregnancy)
No risk factors have been recorded for this pregnancy.

Ethnic Screening

Active Problems
Problems (Active Problems Only)

Pregnancy (SNOMED CT: 128451012, Onset: 10/13/25)
Obese class I (SNOMED CT: 660381000124114, Onset: --)
Fetal ultrasound ventriculomegaly (SNOMED CT: 3514104012, Onset: --)

Procedure/Surgical History
- **Nose** | **Service Date:** 2014

Medications
Medications reviewed and reconciled at today's visit.
aspirin (Aspir 81)**, 81 mg, Oral, Daily**
multivitamin, prenatal (Prenatal 19 (Cypress))**, Daily**

Allergies
No known MEDICATION allergies

Social History

Alcohol
Use: Denies Use
Type: None
Frequency: 1-2 times per year
Average drinks per episode in last year: 0

Electronic Cigarette/Vaping
E-Cigarette Use: Former use, quit more than 90 days ago
Type: Nicotine infused
Use Per Day: None
Use Number Of Years: 3

Home/Environment
Feels Unsafe at Home: No
Safe place to go: Yes

No ethnicities have been recorded.

Genetic Disorders Screening
No genetic disorders have been recorded.

History of Present Illness
Tessa is a 25-year-old G1P0 at 31.4 weeks gestation with pregnancy complicated by fetal ventriculomegaly on ultrasound, presenting for prenatal follow-up with concern about diffuse itching, and sciatica pain.

Pregnancy with dating review and fetal ventriculomegaly
Gestational dating was reviewed per pt request.
Ventricular fluid finding has remained unchanged on most recent ultrasound and considered stable. She has another follow-up ultrasound scheduled for 06/15. Third-trimester labs were reviewed. She reports good fetal movement and denies vaginal bleeding or leaking fluid.

Diffuse itching in pregnancy
For the past 2 to 3 weeks, she has had diffuse pruritus involving the hands, palms, fingers, soles of the feet, back, and legs, without rash. The itching of the feet is worse at night.

She reports sciatica pain during the pregnancy. She had been seeing a chiropractor with some relief, but stopped after changing insurance, which does not cover chiropractic care.

Review of Systems
Genitourinary: Negative for vaginal bleeding. Negative for leaking of fluid.
Musculoskeletal: Positive for sciatica pain.
Skin: Positive for generalized pruritus (itching). Negative for rash.
Denies UCs. Reports good fetal movement.

Constitutional: No fevers, chills, sweats
Eye: No recent visual problems
ENMT: No ear pain, nasal congestion, sore throat
Respiratory: No shortness of breath, cough
Cardiovascular: No Chest pain, palpitations, syncope
Gastrointestinal: No nausea, vomiting, diarrhea
Genitourinary: No hematuria
Hema/Lymph: Negative for bruising tendency, swollen lymph glands
Endocrine: Negative for excessive thirst, excessive hunger
Musculoskeletal: No back pain, neck pain, joint pain, muscle pain, decreased range of motion
Integumentary: No rash, pruritus, abrasions
Neurologic: Alert & oriented X 4
Psychiatric: No anxiety, depression
Immunologic/Allergic: No symptoms

Physical Exam

Vitals & Measurements
T: 36.7 °C (98.1 °F) (Temporal Artery) **HR:** 95 (Peripheral) **BP:** 112/75
SpO2: 95%
HT: 167.64 cm (5 ft 6 in) **WT:** 110.4 kg (243 lbs 7 oz) **BMI:** 39.28

SpO2 details:
Oxygen Therapy: Room air - 05/22/26 14:47

Comments: unable to ask safety question, pt partner was present in room

Sexual
Sexually Active: Yes
Do you think of your sexual orientation as: Straight or heterosexual
What is your current gender identity? Identifies as female

Substance Abuse
Use: Never

Tobacco
Use: Never

Family History
Breast cancer: Grandmother (M).

POC Lab Results
Point-of-Care Lab Results
No qualifying data available

Other Measurements:
-Other Measurements-
BSA (pt care): 2.27 (05/22/2026)
Ideal Body Weight Calculated: 59.3 kg (05/22/2026)

Test	Result	Date
Blood Type	A Positive	12/22/25
Antibody Screen	Negative	
Hgb (1st Trim.)	13.9	
HbsAg	Negative	
HIV	NR	
RPR (1st Trim.)	NR	
Rubella	Immune	
Urine Culture		
GC/CT	Neg/Neg	
Hepatitis C Ab	Negative	
Pap Smear and HPV		
1 HR GTT (1st Trim.)/HgbA1C		
3 HR GTT (1st Trim.)		
TSH		
AST/ALT		
Protein/CrtRatio		
GENETICS		
NT SCAN		
NIPT		
CARRIER SCREENING	Fragile X Carrier	12/22/25
FETAL RH		
IMMUNIZATIONS		
FLU		
TDAP	Given	4/22/26
RSV		
THIRD TRIMESTER		
1 HR GTT (3rd Trim.)	78	4/20/26
3 HR GTT (3rd Trim.)		
Hgb (3rd Trim.)	11.0	4/20/26
RPR (3rd Trim.)	NR	4/20/26
Antibody Screen (3rd Trim)		
GBS		
Other Labs:		

[1]
FH 32cm
FHTs 145bpm

Integumentary:
No rash observed

General: Alert and oriented, well nourished, no acute distress.
Skin: Skin is warm, dry and pink, no rashes or lesions.
HEENT: Normocephalic, ,normal hearing, moist oral mucosa, normal conjunctiva.
Musculoskeletal: Normal range of motion and strength, no edema.

Neurologic: Awake, alert and oriented x 3, no gross focal neurological deficits
Psychiatric: Cooperative, appropriate mood and affect.

Scales And Assessments
Other Screenings:
Numeric Rating Pain Scale: 0 = No pain
Numeric Rating Pain Score: 0
Feeling Down, Depressed, Hopeless: Not at all
Little Interest - Pleasure in Activities: Not at all
PHQ2 Score: 0
Depression Screening Negative: Negative
STEADI Has Fallen In The Past Year: No
STEADI Feel Unsteady Standing/Walking: No
STEADI Worries About Falling: No
STEADI Fall Screening Assessment Result: Screened NOT at risk
Menstrual Status: Menarchal
Pregnancy Status: Currently pregnant

Assessment/Plan
Tessa is a 25-year-old G1P0 at 31.4 weeks gestation with pregnancy complicated by fetal ventriculomegaly presenting today for prenatal follow-up, and evaluation of pruritus in pregnancy.

1. Class 1 obesity

2. Pregnancy
- Scheduled routine prenatal follow-up every 2 weeks until 36 weeks, then weekly.
- Third-trimester labs were reviewed.
- Ordered urine culture- missing from record.

Ordered:
Urine Culture (Urine Culture (RL)), Source: Clean Catch Mid Stream, Routine, Routine, RT, 05/22/26 14:54:00 MST, Nurse Collect, 05/22/26 15:00:00 MST, Pregnancy, SQL Arizona Ambulatory Reference Lab

3. Ventriculomegaly of brain on fetal ultrasound
- Follow-up ultrasound finding: bilateral mild ventriculomegaly, clinically stable from last assessment.
- Additional follow-up ultrasound is scheduled for 6-15.

4. Itching
- Diffuse pruritus in pregnancy raises concern for cholestasis.
- Ordered bile acids and CMP stat; instructed patient to complete labs that afternoon at Sonora Quest if able.

Ordered:
Bile Acids, Total, Pregnancy,Enzymatic (RL), Stat, ST, 05/22/26 15:00:00 MST, Nurse Collect, 05/22/26 15:01:00 MST, Itching, SQL Arizona Ambulatory Reference Lab
Comprehensive Metabolic Panel w/GFR (RL) (CMP (RL)), Blood, Stat, ST, 05/22/26 15:00:00 MST Nurse Collect, Itching, SQL Arizona Ambulatory Reference Lab, 05/22/26 15:01:00 MST

5. Sciatica
Ordered:

Referral AMB., Urgent, Physical Therapy, 31.4 weeks gestation with sciatica pain, Type: Evaluate and Treat, Start: 05/22/26 15:18:00 MST, Sciatica

Sciatica pain
- Ordered physical therapy referral per preference after chiropractic care is no longer covered by insurance.

Coded Diagnoses
Ventriculomegaly of brain on fetal ultrasound (Maternal care for (suspected) central nervous system malformation or damage in fetus, hydrocephaly, not applicable or unspecified, O35.06X0)
Class 1 obesity (Obesity, class 1, E66.811)
Pregnancy (Pregnant state, incidental, Z33.1)
Itching (Pruritus, unspecified, L29.9)
Sciatica (Sciatica, unspecified side, M54.30)

[1] Obstetrics Prenatal Office Visit Note; MCDANIEL NP, ERIKA 04/22/2026 09:16 MST

Signature Line

Electronically Signed on 05/22/2026 15:35 MST

MCDANIEL NP, ERIKA

VILLARREAL, TESSA DIANE
Admin Sex: Female
DOB: 05/25/2000
Continuity of Care Document
Summarization of Episode Note | 05/22/2026 to 05/22/2026
Source: BUMT North Campus OPC - OBGYN
Created: 05/22/2026

Demographics

Contact Information:
6516 E TILMAN CIR, TUCSON, AZ 857081145, US
Tel: (503) 935-6008 (Primary Home)
Tel: (503) 935-6008 (Primary Home)
Tel: (503) 935-6008 (Mobile)
Email: villarreal.tessad@gmail.com
Previous Address(es):
--
Marital Status: Married
Religion: --
Race: White
Previous Name(s):

Villarreal, Tessa Diane
VILLARREAL, TESSA DIANE
Villarreal, Tessa Diane
Date of Death: //
Ethnic Group: Not Hispanic or Latino
Language: English
ID: 1393247, 427344, URN:CERNER:IDENTITY-FEDERATION:REALM:C872F37A-44A8-4D3E-9101-998A7CBD8914:PRINCIPAL:00U11KS4JY6ZXATUG
2772742, 00U11KS4JY6ZXATUG298, 4014746, 41499818, 2949114, 2124766, 334539949, 41944359, 2941545

Care Team

Type	Name	Represented Organization	Address	Phone
primary care physician	PCP, NOPCP	--	--	--

Relationships

No Data to Display

Document Details

Source Contact Info

3838 N Campbell Ave Bldg 2, Tucson, AZ 85719-1454, US
Tel: (520)694-6010

Author Contact Info

05/22/2026 3:55 PM
BUMT North Campus OPC - OBGYN

Recipient Contact Info

--

Healthcare Professionals

No Data to Display

IDs & Code Type Data

Document Type ID: 2.16.840.1.113883.1.3 : POCD_HD000040
Document Template ID: 2.16.840.1.113883.10.20.22.1.1 : 2023-05-01, 2.16.840.1.113883.10.20.22.1.1 : --, 2.16.840.1.113883.10.20.22.1.1 : 2015-08-01, 2.16.840.1.113883.10.20.22.1.2 : 2015-08-01
Document ID: 2.16.840.1.113883.3.6874.1174.999362 : 2292548859
Document Type Code: 2.16.840.1.113883.6.1, 34133-9
Document Language Code: en-US
Document Set ID: --
Document Version Number: --

Primary Encounter

Encounter Information

Registration Date: 05/22/2026
Discharge Date: --
Visit ID: --

Location Information

BUMT North Campus OPC - OBGYN
(Work): 3838 N Campbell Ave Bldg 2, Tucson, AZ 85719-1454, US

Providers

Type	Name	Address	Phone
Attending	MCDANIEL, ERIKA	(Work): 3838 N Campbell AveBldg 2, Tucson, AZ 85719- , US	Tel: (520)694-8888 (Work)
Referring	PCP, NOPCP	--	--

Encounter

AZT AMB FIN # 11398823 Date(s): 5/22/26
BUMT North Campus OPC - OBGYN 3838 N Campbell Ave Bldg 2 Tucson, AZ 85719-1454 US (520) 694-6010
Encounter Diagnosis
Class 1 obesity (Discharge Diagnosis) - 5/22/26
Pregnancy (Discharge Diagnosis) - 5/22/26
Ventriculomegaly of brain on fetal ultrasound (Discharge Diagnosis) - 5/22/26
Itching (Discharge Diagnosis) - 5/22/26
Sciatica (Discharge Diagnosis) - 5/22/26
Attending Physician: MCDANIEL NP, ERIKA
Referring Physician: PCP MD, NOPCP
Encounter Type: Prenatal

Author: MCDANIEL, ERIKA, BUMT North Campus OPC - OBGYN
Last Modified: 05/22/2026 10:17 PM

Allergies, Adverse Reactions, Alerts

No Known Medication Allergies
Author: Mullins, Lisa, BUMT North Campus OPC - OBGYN
Last Modified: 03/24/2026 9:14 PM

Treatment Plan

Extracted from:

Title: Obstetrics Prenatal Office Visit Note Author: MCDANIEL NP, ERIKA Date: 5/22/26

Tessa is a 25-year-old G1P0 at 31.4 weeks gestation with pregnancy complicated by fetal ventriculomegaly presenting today for prenatal follow-up, and evaluation of pruritus in pregnancy.

1. Class 1 obesity

2. Pregnancy
- Scheduled routine prenatal follow-up every 2 weeks until 36 weeks, then weekly.
- Third-trimester labs were reviewed.
- Ordered urine culture- missing from record.

Ordered:
Urine Culture(Urine Culture (RL)), Source: Clean Catch Mid Stream, Routine, Routine, RT, 05/22/26 14:54:00 MST, Nurse Collect, 05/22/26 15:00:00 MST, Pregnancy, SQL Arizona Ambulatory Reference Lab

3. Ventriculomegaly of brain on fetal ultrasound
- Follow-up ultrasound finding: bilateral mild ventriculomegaly, clinically stable from last assessment.
- Additional follow-up ultrasound is scheduled for 6-15.

4. Itching
- Diffuse pruritus in pregnancy raises concern for cholestasis.
- Ordered bile acids and CMP stat; instructed patient to complete labs that afternoon at Sonora Quest if able.

Ordered:
Bile Acids, Total, Pregnancy,Enzymatic (RL), Stat, ST, 05/22/26 15:00:00 MST, Nurse Collect, 05/22/26 15:01:00 MST, Itching, SQL Arizona Ambulatory Reference Lab
Comprehensive Metabolic Panel w/GFR (RL)(CMP (RL)), Blood, Stat, ST, 05/22/26 15:00:00 MST Nurse Collect, Itching, SQL Arizona Ambulatory Reference Lab, 05/22/26 15:01:00 MST

5. Sciatica
Ordered:
Referral AMB., Urgent, Physical Therapy, 31.4 weeks gestation with sciatica pain, Type: Evaluate and Treat, Start: 05/22/26 15:18:00 MST, Sciatica

Sciatica pain
- Ordered physical therapy referral per preference after chiropractic care is no longer covered by insurance.

Future Appointments

Appointment Date: 06/02/2026 01:15:00 PM
Scheduled Provider: NAPPER CNM, AMANDA
Location: AZT NC OBGYN

Appointment Type: OB Est FU

Appointment Date: 06/15/2026 02:30:00 PM
Scheduled Provider:
Location: AZT NC OBGYN
Appointment Type: OB US Mid

Appointment Date: 06/18/2026 02:30:00 PM
Scheduled Provider: MCDANIEL NP, ERIKA
Location: AZT Houghton OBGYN
Appointment Type: OB Est FU

Appointment Date: 06/30/2026 02:30:00 PM
Scheduled Provider: NAPPER CNM, AMANDA
Location: AZT Houghton OBGYN
Appointment Type: OB Est FU

Appointment Date: 07/07/2026 02:00:00 PM
Scheduled Provider: MCDANIEL NP, ERIKA
Location: AZT Houghton OBGYN
Appointment Type: OB Est FU

Appointment Date: 07/14/2026 02:30:00 PM
Scheduled Provider: MCDANIEL NP, ERIKA
Location: AZT Houghton OBGYN
Appointment Type: OB Est FU

Appointment Date: 07/20/2026 04:00:00 PM
Scheduled Provider: NAPPER CNM, AMANDA
Location: AZT Houghton OBGYN
Appointment Type: OB Est FU

Diagnostic Tests Pending

- Urine Culture 5/22/26
- Comprehensive Metabolic Panel w/GFR (RL) 5/22/26
- Bile Acids, Total, Pregnancy,Enzymatic (RL) 5/22/26

Future Scheduled Tests

Radiology:

- US OB Re-Eval Comp 1 Baby 5/18/26
  Author: Banner Health
  Last Modified: 05/18/2026 9:31 PM

Functional Status

No data available for this section

Immunizations

Given and Recorded

Vaccine	Date	Status	Refusal Reason
diphtheria/pertussis, acel/ tetanus adult[1]	4/22/26	Given	

[1]Result Comment: verified by 389569

Medications

Aspir 81

81 mg, Oral, Daily, 0 Refill(s), Signed: 3/24/26 2:15:00 PM MST, Maintenance
Start Date: 3/24/26
Status: Ordered
Medication Dispense Status: Completed
Total Allowed Fills: 1
Fills Dispensed: 0
Author: Mullins, Lisa, BUMT North Campus OPC - OBGYN
Last Modified: 03/24/2026 9:15 PM

Prenatal 19 (Cypress)

Daily, 0 Refill(s), Signed: 3/24/26 2:15:00 PM MST, Maintenance
Start Date: 3/24/26
Status: Ordered
Medication Dispense Status: Completed
Total Allowed Fills: 1
Fills Dispensed: 0
Author: Mullins, Lisa, BUMT North Campus OPC - OBGYN

Last Modified: 03/24/2026 9:15 PM

Mental Status

Mental Status Assessment

Assessment	Assessment Component	Result	Effective Date
PHQ2 Score		0	5/22/26

Problem List

Condition	Confirmation	Course	Effective Dates	Status	Health Status	Informant
Ventriculomegaly of brain on fetal ultrasound Author: MCDANIEL, ERIKA, BUMT North Campus OPC - OBGYN Last Modified: 05/22/2026 9:43 PM	Confirmed			Active Author: MCDANIEL, ERIKA, BUMT North Campus OPC - OBGYN Last Modified: 05/22/2026 9:43 PM		
Class 1 obesity Author: MCDANIEL, ERIKA, BUMT North Campus OPC - OBGYN Last Modified: 05/22/2026 9:42 PM	Confirmed			Active Author: MCDANIEL, ERIKA, BUMT North Campus OPC - OBGYN Last Modified: 05/22/2026 9:42 PM		
Pregnancy Author: NAPPER, AMANDA, Banner Health Last Modified: 03/24/2026 9:29 PM	Confirmed		10/13/25	Active Author: NAPPER, AMANDA, Banner Health Last Modified: 03/24/2026 9:29 PM		

Diagnosis	Diagnosis Type	Effective Dates	Health Status	Clinical Service	Informant
Pregnancy Author: MCDANIEL, ERIKA, BUMT North Campus OPC - OBGYN Last Modified: 05/22/2026 9:42 PM	Discharge Diagnosis	5/22/26		Non-Specified	
Class 1 obesity Author: MCDANIEL, ERIKA, BUMT North Campus OPC - OBGYN Last Modified: 05/22/2026 9:42 PM	Discharge Diagnosis	5/22/26		Non-Specified	
Ventriculomegaly of brain on fetal ultrasound Author: MCDANIEL, ERIKA, BUMT North Campus OPC - OBGYN Last Modified: 05/22/2026 9:43 PM	Discharge Diagnosis	5/22/26		Non-Specified	
Itching Author: MCDANIEL, ERIKA, BUMT North Campus OPC - OBGYN Last Modified: 05/22/2026 9:53 PM	Discharge Diagnosis	5/22/26		Non-Specified	
Sciatica Author: MCDANIEL, ERIKA, BUMT North Campus OPC - OBGYN Last Modified: 05/22/2026 10:17 PM	Discharge Diagnosis	5/22/26		Non-Specified	

Procedures

Procedure	Date	Related Diagnosis	Body Site	Status
Nose Author: Mullins, Lisa, BUMT North	2014			Completed

Procedure	Date	Related Diagnosis	Body Site	Status
Campus OPC - OBGYN Last Modified: 03/24/2026 9:16 PM				

Results

No data available for this section

Vital Signs

Most recent to oldest [Reference Range]:	1
Blood Pressure [91-139/89 mmHg]	112/75 mmHg (5/22/26 2:47 PM) Last Modified: 05/22/2026 2:49 PM Last Modified: 05/22/2026 2:49 PM
SpO2 [90-100 %]	95 % (5/22/26 2:47 PM) Last Modified: 05/22/2026 2:49 PM
Oxygen Therapy	Room air (5/22/26 2:47 PM) Last Modified: 05/22/2026 2:49 PM
Height	167.64 cm (5/22/26 2:47 PM) Last Modified: 05/22/2026 2:49 PM
Weight	110.4 kg (5/22/26 2:47 PM) Last Modified: 05/22/2026 2:49 PM
BMI (Pt Care)	39.28 kg/m2 (5/22/26 2:47 PM) Last Modified: 05/22/2026 2:49 PM
BSA (pt care)	2.27 (5/22/26 2:47 PM) Last Modified: 05/22/2026 2:49 PM
Temperature Temporal Artery [36.5-37.9 DegC]	36.7 DegC (5/22/26 2:47 PM) Last Modified: 05/22/2026 2:49 PM
Peripheral Pulse Rate [50-100 bpm]	95 bpm (5/22/26 2:47 PM) Last Modified: 05/22/2026 2:49 PM

Social History

Social History Type	Response
Electronic Cigarette/Vaping	Electronic Cigarette Use: Former use, quit more than 90 days ago. Type: Nicotine infused. None Use Per Day:. Number of Years: 3. Author: Catalan, Nyssa Last Modified: 05/22/2026 2:49 PM
Smoking Status	Never entered on: 5/22/26 Author: Catalan, Nyssa, BUMT North Campus OPC - OBGYN Last Modified: 05/22/2026 9:49 PM
Sexual Orientation Author: Catalan, Nyssa, BUMT North Campus OPC - OBGYN Last Modified: 05/22/2026 9:49 PM	Do you think of your sexual orientation as: ; Straight or heterosexual
Gender Identity Author: Catalan, Nyssa, BUMT North Campus OPC - OBGYN Last Modified: 05/22/2026 9:49 PM	What is your current gender identity? ; Identifies as female
Birth Sex Author: Banner Health Last Modified: 05/22/2026 9:20 PM	
Sex Representation Author: Banner Health Last Modified: 05/22/2026 9:20 PM	Female
Pregnancy Status	Currently pregnant

Health Concerns

No data available for this section

Implantable Device List

No data available for this section

Hospital Discharge Instructions

Section Author: MCDANIEL, ERIKA, BUMT North Campus OPC - OBGYN, 05/22/2026 10:35 PM

Patient Education

05/22/2026 15:35:08

Healthy Eating Habits During Pregnancy

Healthy Eating Habits During Pregnancy
It's important to develop healthy eating habits while you are pregnant, for you as well as for your baby. Here are some ways to stay healthy.
Aim for a healthy weight
A slow, steady rate of weight gain is often best. After the first trimester, you may gain about a pound a week. If you were overweight before pregnancy, you need to gain fewer pounds. Your healthcare provider can give you a healthy weight goal for your pregnancy.
Don't diet
Now is not the time to diet. You may not get enough of the nutrients you and your baby need. Instead, learn how to be a healthy eater. Start by doing it for your baby. Soon, you may do it for yourself.
Vitamins and supplements
Talk with your healthcare provider about taking these and other prenatal vitamins and supplements.
• Iron makes the extra blood you need now.
• Calcium and vitamin D help build and keep strong bones.
• Folic acid helps prevent certain birth defects.
• Iodine helps the thyroid work right.
• Some vitamins may not be safe to take. Your healthcare provider will tell you which ones to avoid.
Fluids

Drink at least 8 to 10 cups of fluid daily. Your baby needs fluids. Fluids also decrease constipation, flush out toxins and waste, limit swelling, and help prevent bladder infections. Water is best. Other good choices are:
• Water or seltzer water with a slice of lemon or lime. (These can also help ease an upset stomach.)
• Clear soups that are low in salt
• Low-fat or fat-free milk, soy or rice milk with calcium added
• Ice pops or gelatin
Things to avoid
Some things might harm your growing baby. Don't eat or drink:
• Alcohol
• Unpasteurized dairy foods and juices
• Raw or undercooked meat, poultry, fish, or eggs
• Unwashed fruits and vegetables

• Prepared meats, like deli meats or hot dogs, unless heated until steaming hot
• Fish that are high in mercury, like shark, swordfish, king mackerel, tilefish, and albacore tuna
Things to limit
Ask your healthcare provider whether it's safe to eat or drink:
• Caffeine
• Artificial sweeteners
• Organ meats
• Certain types of fish
• Fish and shellfish that contain mercury in lower amounts, like shrimp, canned light tuna, salmon, pollock, and catfish

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Adapting to Pregnancy: Third Trimester

Adapting to Pregnancy: Third Trimester
Although common during pregnancy, some discomforts may seem worse in the final weeks. Simple lifestyle changes can help. Take care of yourself.
Limiting leg problems
Ways to ease leg problems:
• Wear compression socks all day.
• Don't wear snug shoes or clothes that bind, such as tight pants and socks with elastic tops.
• Whenever possible, sit with your feet and legs raised.
Caring for your breasts
Here are some tips that can help:
• Wash your breasts with mild body soap and warm water. Dry with a clean towel.
• Wear a nursing bra for extra support. It can also hide any leaks from your nipples.
Controlling hemorrhoids
Ways to prevent hemorrhoids include:
• Eating foods that are high in fiber. Also exercise and drink enough fluids. This will reduce constipation and hemorrhoids.
• Sleeping and napping on your side. This limits pressure on the veins of your rectum (lower part of your bowel).
• Trying not to stand or sit for long periods.
Controlling back pain
As your body changes during pregnancy, your back must work in new ways. Back pain has many causes. Physical changes in your body can strain your back and its supporting muscles. This can affect how your muscles and joints work together. All of these changes can lead to pain.
Pain may be felt in the upper or lower back. Pain is also common in the pelvis. Some pregnant women have sciatica. This is pain caused by pressure on the sciatic nerve, which runs down

the back of the leg. Ice or heat may help. Your provider may advise massage therapy or a chiropractor. Sleep on your left side with a pillow between your knees. Ask your provider for tips and exercises to help control your back pain.

Tips to help you rest

Good rest and sleep will help you feel better. Here are some ideas:

- Massage your shoulders, neck, or back.
- Limit the errands you do each day.
- Lie down in the afternoon or after work for a few minutes.
- Take a warm bath before you go to sleep.
- Drink warm milk or teas without caffeine.
- Don't drink coffee, black tea, and caffeinated soft drinks.

Stopping heartburn

- Don't eat spicy, greasy, fried, or acidic foods.
- Eat small amounts more often. Eat slowly.
- Wait 2 hours after eating before lying down.
- Sleep with your upper body raised 6 inches.

Managing mood swings

Ways to manage mood swings include:

- Knowing that mood changes are normal.
- Exercising often, but also getting plenty of rest.
- Limiting stress. Talk to your doctor about ways to manage stress.

Dealing with urinary frequency

Here are some tips if you have to urinate often:

- Drink plenty of water all day. You may want to limit water before bedtime so you don't have to get up more at night.
- Limit coffee, black tea, and caffeinated soft drinks.

How daily issues affect your health

Many things in your daily life affect your health. These can include transportation, money problems, housing, access to food, and child care. If you can't get to medical appointments, you may not receive the care you need. When money is tight, it may be difficult to pay for medicines. And living far from a grocery store can make it hard to buy healthy food.

If you have concerns in any of these or other areas, talk with your care team. They may know of local resources to assist you.

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Skin Changes During Pregnancy

Skin Changes During Pregnancy

Pregnancy is a time of great change for your body—and that can include changes in how your skin looks and feels, too. Below you will find helpful information on the many ways being pregnant can affect your skin. Often these changes are due to the increased hormones your body makes during this time.

If you have any questions about your skin during your pregnancy, contact your care team. They can examine you and address any concerns you have.

Acne

• What it is: Pimple breakouts can occur during pregnancy, even if you never had acne before. And any acne issues you may have had before can get a bit worse now.

• When it occurs: First trimester

• Why it happens: Higher levels of the hormone progesterone occur in the first trimester. This makes your glands produce more oil. The extra oil can clog up your pores and lead to bacteria buildup, which causes acne.

• What you can do: Many acne medicines are not safe to take during pregnancy. Talk with your care team to see what they advise. In the meantime, treat your skin gently by:

? Washing your face with a mild cleanser in the morning at night. Don't scrub your skin.

? Rinsing with lukewarm water, not hot water

? Using oil-free moisturizers and makeup

Changes in moles

• What they are: Any existing moles you have may get darker or larger as your belly and breasts expand during pregnancy. You may also have new moles.

• When they occur: Any time during pregnancy

• Why they happen: Experts think this is due to hormonal changes

• Are they serious: These changes are often harmless. But any changes in moles should be checked by your care team.

• How long do they last: These changes often go away once you give birth.

• What you can do: Do regular mole checks. Call your doctor if you notice changes in mole size, shape, or color.

Cholestasis

• What it is: Very itchy or dry skin that occurs without a rash. This may occur on your palms or the bottom of your feet. But it can happen in other parts of the body too.

• When it occurs: Third trimester, but may start earlier

• Why it happens: This is a liver condition that occurs when you are pregnant. The liver makes a substance called bile. This normally goes from the liver to the small intestine. But with this condition the bile is blocked. It goes into your skin instead.

• Is it serious: It may raise your risk for preterm birth. In rare cases it can cause stillbirth.

• How long does it last: It often goes away a few days after you give birth. But you may have it in future pregnancies too.

• What you can do: Call your doctor if itching is severe or does not go away. You may have a blood test done for diagnosis. If you have cholestasis, you and your fetus will be watched closely for the rest of the pregnancy. Your care team may advise an early delivery by cesarean section. And they may advise medicine for the itching.

Darkening of the skin (hyperpigmentation):
• What it is: Parts of the skin may become darker. This often occurs around the nipples, areolas and inner thighs. Or you may have a dark vertical line on your belly, from the navel to the pubic area (called linea nigra).
• When it occurs: By the second trimester
• Why it happens: High levels of certain hormones like progesterone and estrogen during pregnancy
• Is it serious: No
• How long does it last: Usually fades after delivery
• What you can do: Use sunscreen and protect your skin from the sun.
Melasma (pregnancy mask)
• What it is: Patches of darkened skin, often on the cheeks and forehead. The patches can look a bit like a mask.
• When it occurs: Second and third trimesters
• Why it happens: Higher levels of melanin
• Is it serious: No, but you may not like how it looks
• How long does it last: The patches often fade away after you give birth. But some areas may never fully go away.
• What you can do: Protecting your skin from the sun can stop these dark patches from getting worse. Wear a wide-brimmed hat and use sunscreen even when it's cloudy outside. Talk with your doctor if the condition doesn't go away after delivery. They can advise you about skin-lightening creams, oral medicine, and treatments such as chemical peels and laser or light therapy.
Palmar erythema
• What it is: Red, blotchy spots on the palms of the hands. They may be itchy. These spots are often first seen on the bottom (heel) of the palm. But they can also spread to the fingers.
• When it occurs: First trimester
• Why it happens: In most cases, it's caused by high estrogen levels during pregnancy.
• Is it serious: Not serious, unless it is linked to an underlying health condition
• How long does it last: Usually fades after pregnancy, once estrogen levels go back to normal
• What you can do: Let your doctor know if your palms stay red for a few days, or if they are very itchy. Your doctor can make sure this is not caused by another condition. And they can advise you on anti-itch medicines that are safe to take.
Pemiphoid gestationis
• What it is: A rare condition that causes blisters on the belly. Some people may have blisters over a larger part of the body.
• When it occurs: Second or third trimester, or in some cases right after delivery
• Why it happens: Experts think it's an autoimmune disorder
• Is it serious: It can be. You may be at a higher risk of pregnancy problems like preterm birth. Or of having a baby who is smaller than average. In some cases the baby may be born with blisters too. But these may only last for a few weeks.
• How long does it last: The blisters will go away over time. But this condition can happen again during a future pregnancy.
• What you can do: If you have blisters, see your doctor for a diagnosis. They will watch you and your fetus closely. They may also advise medicines to take for the blisters.

Prurigo of pregnancy
• What it is: Very tiny, itchy raised bumps that can appear anywhere on the body
• When it occurs: Anytime during pregnancy
• Why it happens: It may occur because of changes in the immune system during pregnancy
• Is it serious: No, but it can be very uncomfortable
• How long does it last: Often lasts for a few months after giving birth
• What you can do: Call your doctor for anti-itch medicines and other options such as antihistamines.
PUPPP (Pruritic urticarial papules and plaques of pregnancy)
• What it is: A red, itchy rash made up of small, raised red bumps. It often starts on the belly. It may spread to the breasts, buttocks, and thighs.
• When it occurs: Later in pregnancy
• Why it happens: Experts don't know
• Is it serious: No, but the itching can be uncomfortable
• How long does it last: Usually goes away after you give birth
• What you can do: If the itching is bothering you, see your doctor. They can prescribe some anti-itch medicine for relief.
Skin tags
• What they are: Small, soft, flesh-colored growths. They often appear in skin folds. Or in places where other skin or clothing rubs against the skin.
• When they occur: Anytime during pregnancy
• What causes them: Hormonal changes during pregnancy
• Are they serious: No
• How long do they last: They may go away after delivery, but sometimes they last for a while afterward.
• What you can do: Don't try to remove them yourself. This can lead to bleeding or infection. If they don't go away after your baby is born, or if they become irritated, see your doctor. They can remove them safely.
Stretch marks
• What they are: Pink, reddish, or dark streaks that may appear on the belly, breasts, hips, or thighs. They can occur as the skin in these areas stretches during pregnancy. They may also be itchy.
• When they occur: Second and third trimesters
• Why they happen: Stretch marks can happen any time there is a quick weight gain or weight loss. They can also be caused by hormonal changes.
• Are they serious: No
• How long do they last: They generally fade in color over time, but may not go away
• What you can do: Talk with your doctor. There are some treatments available, such as medicated creams, light and laser treatments, and microdermabrasion. But not all are safe to use when pregnant.
Spider veins
• What they are: Tiny red veins that typically appear on your face, arms, neck, and upper chest. They are painless.
• When they occur: First or second trimester
• Why they happen: Hormonal changes and increased blood circulation

• Are they serious: No
• How long do they last: May fade after pregnancy but sometimes can be permanent
• What you can do: If spider veins don't go away after your pregnancy, talk with your doctor. Laser surgery to remove them may be an option.
Varicose veins
• What they are: Large, blue, swollen veins that appear on your legs. They can also occur in the genital area or the rectum. They may feel sore, itchy, and uncomfortable.
• Why they occur: They are often hereditary. So if your mother or grandmother had them, you are more likely to have them.
• When they happen: Second or third trimester
• Are they serious: Not often serious, but they can be painful
• How long do they last: In most cases, they go away a few months after your baby is born
• What you can do: Call your doctor if you have any concerns. To help ease your symptoms:
? Don't stand or sit for a long time
? Don't sit with your legs crossed
? Keep your legs raised (elevated) when resting
? Wear compression stockings
? Stay active

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Goals

No data available for this section

Reason for Referral

No data available for this section

Clinical Note

No data available for this section

Patient Care team information

Care Team Personnel

Name: PCP MD, NOPCP
Member Role: Primary Care Physician

Author: Banner Health
Last Modified: 04/22/2026 4:16 PM

Family History

No data available for this section

Insurance Providers

Guarantor name: TESSA DIANE VILLARREAL

Health Plan Information #: 1
Payer: TRICARE WEST TRIWEST
Payer Identifier: NA
Member Number: 02261091601
Group Number: 0
Subscriber Identifier: 02261091601
Relationship to Subscriber: Self
Coverage Type: TRICARE Standard - Fee For Service
Coverage Verification Date: NA
Telecom: (877) 226-8749
Address: PO BOX 202160
FLORENCE, SC 29502-2160 US

Health Plan Information #: 2
Payer: BCBS OUT OF STATEs
Payer Identifier: CERN000093
Member Number: OER201148335
Group Number: NA
Subscriber Identifier: OER201148335
Relationship to Subscriber: Mother
Coverage Type: Blue Cross/Blue Shield
Coverage Verification Date: 5/16/26
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